جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : اعتماد محمد عيد اسماعيل
المحافظة : الجيزه **الحي / المدينة :** امبابه **العنوان :** منيره
رقم البطاقة : 26805272200285 **رقم التليفون :** 01115505272 **تاريخ تسجيل الإستمارة :** 23-10-2026
العمر(تاريخ الميلاد) : 27-05-1968 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اعتماد محمد عيد اسماعيل
National ID: 26805272200285

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	الأستاذه / اعتماد محمد عيد اسماعيل	Request No.	13002090
Patient No.	26805272200285	Request Date	29/05/2024
Path No.	1797/24	Verification Date	23/06/2024
Sex	Female	Financial Category	اورام دار السلام - مبادره
Age	56 Y	Department	Out Patient
Referreing Doc.		Registered by	Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonim.
Dr. Sagda Elkady.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Name: اعتماد محمد عيد اسماعيل **MR. :** 4053	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

Metastatic breast cancer to lung and lymph nodes (2/8, PR 3/8, HER2-negative) was treated with 27 weeks of taxol. A CT scan in December 2025 showed no evidence of metastases, prompting a request for new chemotherapy.

A CT scan in June 2026 revealed a right upper quadrant mass measuring 2.4 × 2.5 cm. A PET-CT was ordered to assess the therapeutic plan and the possibility of surgery.

The PET-CT performed on 21 June 2026 demonstrated only a 1 mm positive right breast nodular lesion, with otherwise no metastatic disease. The case was discussed at multidisciplinary team (MDT) meeting for potential surgical intervention, and a surgical clinic consultation was scheduled.

During the surgical clinic assessment on 14 July 2026, a new breast surgical management (BSM) plan was formulated and documented for MDT review.

The surgical evaluation noted a 3 × 4 cm mass located at the 10 o'clock position, with no skin edema or inflammatory signs.

تحريرا في ,

التاريخ :	**20-09-2026**	**الوقت :**	

الطبيب المعالج : ندى حسن صلاح كامل

يعتمد ,

Patient Name	: الأستاذه / اعتماد محمد عيد اسماعيل	Request No.	: 13002090
Patient No.	: 26805272200285	Request Date	: 29/05/2024
Path No.	: 1797/24	Verification Date	: 02/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 56 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast 10 O'clock mass.

Nature of Specimen: Tru-cut biopsy, collected at 28/05/2024, 1:15 pm.

Gross Examination:
- Multiple grayish white tissue cores, up to 1.2 cm long, totally submitted.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
* Ductal carcinoma in situ: Not detected.
* Perineural invasion: Not detected.
* Angioinvasion: Not detected.
* Additional findings: Focal necrosis.

Diagnosis:
Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade III.
- For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Gamal Abd-El-Aty.
Dr. Esraa Magdy.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : اعتماد محمد عبد اسماعيل **Sex:** Female
3- Medical number: 83209626
Old Med : 1688-2024
2-ID number: 26805272200285
Patient Age : 56 Y

0 0

Family history	Family History -ve	**Phone**	Phone 01115505272	**BMI**	BMI	**REASON OF MDT**	REASON OF MDT NEW
marital status	Marital Status	**NRS Score**	NRS Score	**GROUP**	GROUP II	**Date of MDT**	Date Of MDT 29/06/2024
Co morbidity	Co Morbidity dm,htn	**Menopausal status**	Menopausal Status POSTMENOPAUSAL	**Initial STAGE**	Intial STAGE		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC BX COMMENT: giii	2/8 ER COMMENT:	3/8 PR COMMENT:	0 Her 2 Neu COMMENT:	SISH COMMENT :	50

T	N	M
2	3b	lung/ln

Sonomammography

PATIENT HISTORY

MBC to mediastinal LN & lung
bone scan
lab

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

to start taxol x 3 cycles then reass

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/	**RADIOLOGY**	RADIOLOGY د/
SURGERY	SURGERY	**PATHOLOGY**	PATHOLOGY
Psychiatric	Psychiatric	**Nutrition**	Nutrition

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name: Eatemad Mohamed Eid Ismail	Date : 23/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 57 Year(s)
Medical ID: 1688/2024	Pat. ID: 54271

MULTISCLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- Right basal atelectatic band
- No pulmonary nodules, masses, consolidation, collapse or cavitary lesions.
- Few enlarged benign looking mediastinal lymph nodes.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No focal lesion or dilated biliary radicles.
- Normal C.T appearance of the spleen, both kidneys , adrenal glands, pancreas, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **No pulmonary nodules, masses or cavitary lesions.**
- **No hepatic focal lesions**
- **No abdominal or pelvic collections**

DR/AYA GABR

Mammogram
[[DOCTYPE:MAMMOGRAM]]


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٢٠٢١/٠٣
ربة منزل
انثى
مسلمة
ارملة
١
البطاقة سارية حتى ٢٠٢٨/٠٣/٠٨


جمهورية مصر العربية
بطاقة تحقيق الشخصية
اعتماد
محمد عيد اسماعيل
٩٢ش محمودالجميل-ش جمال حمزه-منيرة
امبابه - الجيزه
٢٦٨ ٠٥ ٢٧ ٢٢ ٠٠٢ ٨٥
IU4332552

صلية
تدعيم

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Patient's Name:	**I'timad Mohammed Eid Ismail**
Date:	**2026-08-12**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Right breast lump with no available previous data.

Findings:

Right breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- UOQ globular asymmetry with no suspicious microcalcifications
- **Ultrasound** demonstrated 11 o'clock, 5 cm from nipple, 7 mm deep to the skin irregular anti-parallel heterogenous hypoechoic solid mass lesion with indistinct margins and minimal internal vascularity on color Doppler study measuring 39 x 28 mm.
- Few similar anterior masses are noted at the same line, the largest measuring 12 x 13 mm seen 2.5 mm, 8 mm from the index and 11 mm deep to the skin. Anteriorly related prominent ducts are noted reaching the nipple.
- Normal skin/nipple/areola complex.
- One pathological lymph node seen axillary level 1 measuring 6 x 7 mm. no pathological infra or supra-clavicular lymoh nodes

Left breast.

- Heterogeneously dense breast parenchyma that may obscure small lesions. (ACR c)
- no masses speculations, masses, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated condensation of glandular tissue at 7-8 o'clock middle third with no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion:-

Right breast (BIRADS 4c): UOQ suspicious process as described.

Left breast (BIRADS 1): Normal study.

Recommendation:-

For biopsy & histopathological correlation.

Best Regards,

DR. Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routine screening	0 %

[[DOCTYPE:PET_SCAN]]
PET Scan

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : I'timad Mohammed Eid Ismail	Date : 21-06-2026 09:19
Branch : Nasrcity	Medical No. : 398097

Whole body (FDG) PET/CT Study:

Clinical history:

- 58-year-old female patient with history of managed right breast cancer and received chemotherapy ended in June 2025.
- PET/CT is requested for status evaluation.

Technique:

For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed using GE Discovery IQ simultaneously and used for attenuation correction, anatomical localization and diagnosis. Max. variant of SUV; a Semi-quantitative analysis would be done for selected ROI.s and the normal threshold is **< 3.0**

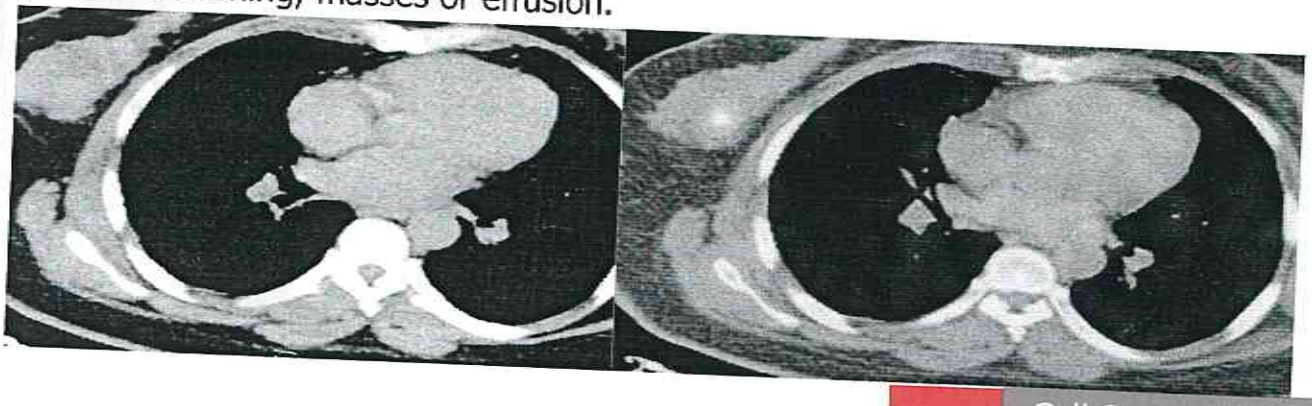
PET/CT findings:

Head and neck:

- Bilateral symmetrical tracer distribution is seen within the brain parenchyma with no abnormal metabolic activity or definite SOLs could be detected.
- Normal appearance of soft tissues and glandular structures of head & neck.
- No pathologically enlarged or tracer-avid cervical or supraclavicular LN.s.

Chest:

- **Metabolically active ill-defined right breast nodular lesion is noted measuring 1[illegible] mm with SUV max 4.1.**
- No current significant metabolically active lung nodules or parenchymal infiltrates.
- No enlarged or tracer-avid mediastinal, hilar or axillary LN.s.
- No pleural thickening, masses or effusion.



[[DOCTYPE:PET_SCAN]]
PET Scan

NILE SCAN & LABS

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Name : I'timad Mohammed Eid Ismail	Date : 21-06-2026 09:19
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Abdomen and Pelvis:

- Average-sized liver showing homogenous CT-texture and tracer uptake pattern with no definite focal lesions (on non-contrast CT basis.
- Unremarkable spleen, kidneys, adrenals, pancreas and pelvic organs.
- No definite gastric or bowel thickening, mural masses (on non-oral contrast CT basis) or tracer-avid foci could be detected.
- No pathologically enlarged or tracer-avid abdomino-pelvic LN.s.
- No evidence of ascites, free or encysted abdomino-pelvic fluid collections.

Musculoskeletal & BM:

- No any pathological sclerotic/lytic bony lesions or abnormal tracer-avid foci could be detected over the surveyed skeleton.

Impression:

Whole-body (FDG) PET/CT study is showing:

- **Metabolically active ill-defined right breast nodular lesion mostly of active tumor biology therein.**

- **The rest of the surveyed body show no scintigraphic evidence of any other significant metabolically active lesions.**

Best regards,

Dr. Ahmed dowydar, MSc.

Dr. Alaa Nagi, MD.
Nuclear Medicine

NILE DIAGNOSTIC CENTER


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٢٠٢١/٠٣
ربة منزل
انثى
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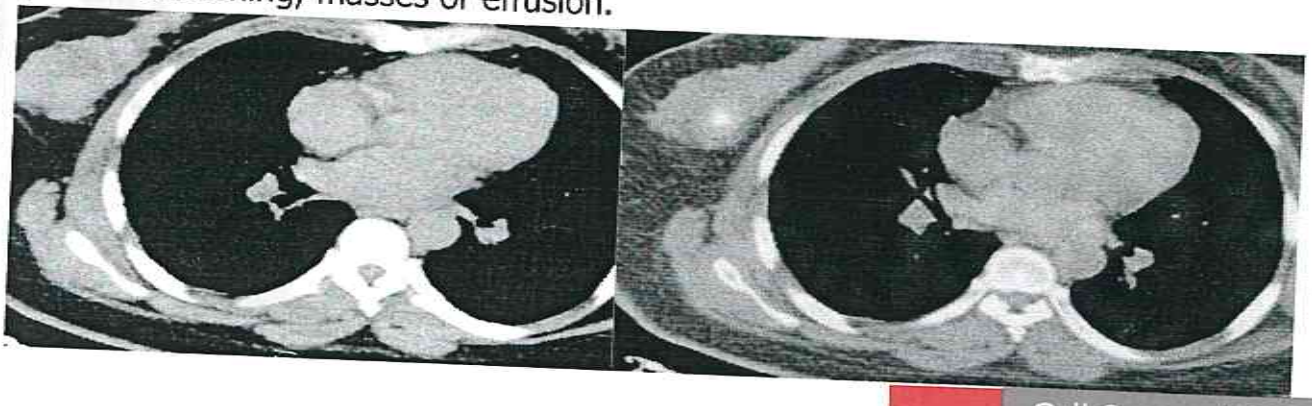
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